VUGA NDAKUMVA INITIATIVE

# CHAPTER 05
# Domestic Violence and Intimate Partner Violence

Recognizing abuse, understanding coercive control, supporting survivors safely, and finding appropriate help.

Updated edition - November 2026
Educational resource | English | For adults, students, educators and community supporters

Reader safety note: You do not need to disclose personal experiences to use this chapter. If your device may be monitored, opening GBV information may create additional risk. Consider a safer device or trusted service. For immediate danger, seek emergency assistance when safe.

Publication status: Evidence-informed educational draft. Requires specialist safeguarding, legal and clinical review before public distribution.

### Learning goals

After studying this chapter, readers should be able to distinguish domestic violence from intimate partner violence; recognize physical, sexual, psychological, economic, and digital abuse; identify coercive control and escalation warning signs; understand why leaving or disclosing abuse may be difficult; offer nonjudgmental first-line support; and locate appropriate help in Rwanda.

## 01 Understanding domestic violence

Domestic violence describes abusive behavior occurring within domestic or family relationships. Depending on legal definitions and setting, it may involve spouses, partners, former partners, parents, adult children, siblings, or other household or family members. It can include physical harm and patterns of threats, intimidation, sexual coercion, controlling behavior, or economic restrictions.

Intimate partner violence (IPV) refers to behavior by a current or former intimate partner that causes physical, sexual, or psychological harm, including acts of aggression, sexual coercion, psychological abuse and controlling behaviors. IPV can occur within marriage or dating relationships, including after separation. It is not limited to couples who live together. [1,2]

Many domestic violence incidents are gender-based, especially where violence enforces unequal gender power or expectations; however, the terms domestic violence and GBV are not identical. Anyone affected by abuse deserves support. Population-level evidence shows that women and girls are disproportionately affected by intimate partner and sexual violence. [1]

### Violence is different from ordinary disagreement

Two people may disagree, become frustrated, or need time apart without committing abuse. Abuse involves conduct that harms, threatens, coerces, intimidates, or restricts another person’s autonomy. A repeated pattern of surveillance, humiliation, isolation, or punishment for saying “no” is concerning even when there are no visible injuries. An isolated incident of severe violence is also serious; it need not recur to warrant help.

Remember: Violence is a choice made by the person using abusive behavior. Stress, alcohol, jealousy, poverty, family difficulties, or a survivor’s decision do not justify abuse.

## 02 Forms of violence within relationships

Physical violence Physical violence includes hitting, slapping, kicking, choking or strangulation, pushing, restraining, using a weapon, throwing objects to injure, or preventing a person from escaping. Serious harm may occur without visible marks. A person subjected to pressure on the neck requires urgent medical evaluation because dangerous complications can occur even when symptoms initially seem minor.

### Sexual violence and reproductive coercion

Sexual violence includes any sexual act or contact without freely given agreement, including within marriage or a long-term relationship. Threats, force, fear, intoxication, exploitation of authority, and other forms of coercion can undermine consent. Reproductive coercion may involve sabotaging contraception, pressuring someone about pregnancy outcomes, or restricting reproductive healthcare choices.

### Psychological and emotional abuse

This may involve repeated threats, humiliation, insults, intimidation, blaming a person for the abusive behavior, threatening to harm children or loved ones, or making the person doubt their perceptions. Persistent isolation from friends, family, education, or work can also function as control. The purpose or effect may be to create fear and dependence.

### Economic and digital abuse

Economic abuse may include preventing someone from earning income, taking wages, hiding essential documents, controlling access to food or transport, or creating debt in the other person’s name. Digital abuse may include monitoring messages, demanding passwords, tracking location, impersonation, harassment, or threatening to publish private images. Technology can make controlling behavior continue after physical separation.

## 03 What is coercive control?

Coercive control is an ongoing pattern of intimidation, isolation, monitoring, restrictions, or punishment that progressively reduces a person’s freedom and ability to make choices. A person may appear to “agree” with demands because refusal could trigger threats or harm. The pattern often matters more than any single action.

Example behavior Why it is concerning

Requiring constant location updates May create surveillance, fear, and loss of independence

Preventing visits to trusted friends May isolate the person from support

Threatening to take children away May use parenting fears as a means of coercion

Taking all wages or IDs May reduce access to basic needs and safe options

Threatening self-harm to prevent a May impose guilt and pressure; threats still require appropriate safety breakup response

Not every request, shared financial decision, or relationship disagreement is abuse. Consider context, freedom to decline, retaliation, repeated patterns, and whether one person is afraid of the consequences of saying no.

## 04 Recognizing warning signs and escalation

Warning signs include increasing monitoring, repeated threats, humiliating comments, forced sex, physical assaults, threats with weapons, stalking, isolation, and interference with work or medical care. A person may minimize danger because the behavior has developed gradually, or because periods of kindness alternate with abuse.

Particularly concerning signals include threats to kill, strangulation, access to weapons, forced sex, escalating frequency or severity of violence, stalking after separation, or a survivor expressing fear of imminent serious harm. These signals call for urgent professional safety assessment where possible. Neither an online checklist nor a community volunteer should attempt to predict danger with certainty.

Immediate priority: If there is immediate danger, prioritize reaching a safer place and contacting emergency assistance when it is safe to do so. Do not confront the suspected abusive person or advise a supporter to intervene physically.

## 05 Effects on health, dignity, and daily life

Domestic and intimate partner violence can cause injury, chronic pain, reproductive-health problems, anxiety, depression, traumatic stress, sleep disruption, reduced school or work participation, and financial insecurity. Not every survivor experiences all these outcomes. Survivors may also show resilience, maintain responsibilities, or appear calm while facing significant risk. [1]

Children exposed to violence in the household may experience fear, sleep or concentration difficulties, school disruption, and emotional or behavioral problems. Their needs must be assessed independently, with appropriate child protection procedures. Exposure does not predetermine their future behavior or wellbeing.

### Why survivors may stay, return, or delay seeking help

Reasons can include threats of retaliation, concern for children, financial dependence, housing insecurity, social pressure, disability-related barriers, lack of confidential services, immigration or documentation concerns, and emotional attachment. Leaving a controlling relationship can sometimes increase short-term danger. A survivor should never be blamed for staying, returning, or asking for help more than once.

## 06 Risk factors are not excuses

Research identifies influences at individual, relationship, community, and societal levels. Inequitable gender norms, acceptance of violence, prior exposure to abuse, harmful alcohol use, and weak institutional accountability can be associated with greater risk. No single factor explains an individual abusive act; many people facing the same difficulties never use violence. [1,4]

Prevention requires action beyond individuals: respectful relationship education, accessible services, safe environments, economic opportunity, accountable institutions, and gender-equitable norms. WHO’s RESPECT framework presents seven complementary prevention strategies. [4]

## 07 Safe, survivor-centred first-line support

WHO first-line support emphasizes meeting immediate emotional and physical needs, ongoing safety, and continuing support. The LIVES framework is useful for trained helpers and can also inform respectful responses by trusted supporters. [2,3]

### Element Practical approach

L - Listen Listen patiently, without judgment or repeated requests for details

I - Inquire about needs Ask what the person needs now, privately and without pressure

V - Validate Communicate that the abuse is serious and the survivor is not to blame

E - Enhance safety Ask about immediate safety and link to skilled help rather than imposing a plan

S - Support Explain available care, referral, and practical options; respect choice and confidentiality limits

### Helpful and unhelpful responses

More supportive Potentially harmful

“Thank you for telling me. What would help you feel “Why didn’t you leave earlier?” safer?”

“We can explore options; you decide what feels safe.” “You must report right now, whatever the risk.”

“I will explain what I can and cannot keep “I promise nobody will ever know.” confidential.”

“You do not have to describe every detail to me.” “Tell me exactly everything that happened.”

Supporters should not investigate the incident, promise outcomes, attempt couples mediation where coercion or violence is present, or contact an abusive partner without the survivor’s informed and safe participation. Child protection and mandatory reporting requirements may affect confidentiality; professionals must explain applicable limits.

## 08 Safety planning: principles, not a fixed script

Safety planning should be individualized, ideally with a trained GBV service provider, because apparently ordinary advice can increase risk. A plan may consider where the person can go in an emergency, trusted contacts, safe transport, medical needs, children’s needs, documents, money, and whether communication devices are monitored. The survivor chooses what is appropriate for their circumstances.

Do not advise a person to save a visible list of “escape steps” on a monitored phone, secretly record someone in ways that may be unsafe or unlawful, or confront the abusive person to announce a plan. Clearing browsing history does not always remove traces of activity. Where appropriate, a safer phone or a trusted professional service may reduce exposure.

### After separation

A former partner may continue harassment, tracking, threats, or economic coercion. Support should consider digital privacy, safe communication, childcare and school arrangements, and legal assistance. Decisions about protection measures should be discussed with qualified local providers, not determined by a generic online guide.

## 09 Health care and urgent situations

Medical assessment is important after injuries, strangulation, sexual assault, severe distress, or new concerning symptoms. After sexual violence, timely professional care can address injuries, sexually transmitted infection prevention, emergency contraception where appropriate and available, HIV post-exposure prophylaxis when clinically indicated, and forensic options. Some interventions are time-sensitive; do not wait for legal decisions before seeking medical advice. [2,3]

The patient should be treated with dignity and informed of what examinations, documentation, and reporting procedures may involve. Whether reporting is voluntary or required depends on applicable law and safeguarding responsibilities; providers should explain the situation clearly.

Do not use this handbook to make clinical decisions. Symptoms such as difficulty breathing, severe bleeding, loss of consciousness, head injury, or concerning signs after neck compression need emergency assessment. A qualified health professional must evaluate treatment needs.

# 10 Accessing help in Rwanda

Rwanda’s Isange One Stop Centre model brings together medical, psychosocial, investigative, and legal services for GBV survivors and people affected by child abuse. MIGEPROF describes centres nationwide, while the Rwanda Investigation Bureau publishes current contact lines. UNHCR cautions that visiting an Isange centre may involve reporting to law enforcement; survivors should ask about confidentiality and reporting processes. [5,6,7]

### Official contact Purpose

3029 Isange One Stop Centre (RIB contact page)

3512 Reporting domestic or gender-based violence (RIB)

116 Reporting child abuse (RIB)

166 Emergency or crime-related information (RIB)

9059 MIGEPROF assistance hotline (ministry site)

These numbers are published by the indicated authorities as of the chapter’s November 2026 source review. Availability, operating procedures and eligibility should be checked before relying on them. VUGA NDAKUMVA should maintain a verified referral directory and never imply a guaranteed response time. [5,6]

# 11 Practical scenarios: recognizing what matters

Scenario A - Financial restrictions in a household A married woman earns an income, but her partner confiscates her wages and threatens violence if she asks to pay for transport to work. This may involve economic abuse, threats, and coercive control. A helpful response focuses on confidential discussion of needs, financial safety, and options for trusted assistance - not on demanding that she confront her partner.

### Scenario B - Student relationship and constant monitoring

A university student receives repeated demands for screenshots of messages and live location. Refusal leads to threats of public humiliation. Monitoring and threats are serious warning signs even without physical violence. Supporters should respect the student’s choices and avoid sending visible messages that could reveal the conversation to the controlling partner.

### Scenario C - A man experiencing abuse

A man reports repeated assault and threats by a partner but fears he will be mocked if he seeks help. His experience should be heard without ridicule or assumptions about who can be a survivor. He deserves an appropriate safety assessment and access to relevant services.

### Scenario D - Children in an unsafe home

A child hears threats and sees a caregiver pushed during arguments. An educator notices sudden attendance and concentration problems. The child may need a safe, age-appropriate safeguarding response. Teachers should follow designated reporting procedures rather than interview the child repeatedly or promise secrecy.

### Scenario E - Risks after separation

A former partner repeatedly appears at a person’s workplace and threatens to take away their children. This may indicate stalking and coercion after separation. Trained GBV and legal providers can help consider appropriate safety and protection options.

## 12 Guided learning activities

Activity 1 - Identify the pattern Read Scenario B. List three behaviors that could indicate coercive control. Then ask: would the student be free to refuse without fear? Sample answer: demands for private messages, constant location surveillance, and threats of humiliation; fear of retaliation suggests the requests are not freely negotiated boundaries.

### Activity 2 - Practice a supportive response

Imagine someone says: “My partner threatens me whenever I try to visit my friends.” Write a two-sentence response that validates their experience without instructing them what to do. Sample: “That sounds frightening, and you deserve to be treated with respect. Would you like to talk about what kind of support would feel safe for you?”

### Activity 3 - Separate risk factors from blame

Consider a household with unemployment and harmful alcohol use. Explain why these factors may be relevant to prevention but do not excuse violence. Sample answer: context can influence risk, but responsibility rests with the person using violence.

### Activity 4 - Privacy and digital safety

Identify why texting detailed safety instructions to a monitored phone could be dangerous. Sample answer: the abusive person may see the messages, increase surveillance, or retaliate. Safer communication options require careful discussion with a qualified supporter.

### Activity 5 - Referral mapping

Identify two official services from the Rwanda contact table and explain their published roles. Do not make test calls to crisis lines as an exercise. A community organization should verify contacts through official channels before publishing locally tailored materials.

## 13 Knowledge assessment with explained answers

1. Is intimate partner violence limited to married couples?

Answer: No. It can occur in current or former dating or intimate relationships, whether or not partners live together.

2. Can someone experience abuse without physical injuries?

Answer: Yes. Psychological, sexual, economic, and digital harm or coercion may be present.

3. Is jealousy evidence of love when it involves constant monitoring?

Answer: No. Surveillance and threats can indicate coercive control.

4. Does economic hardship justify physical violence?

Answer: No. Circumstances do not remove responsibility for abusive choices.

5. Should a supporter contact the suspected abusive partner to settle the matter?

Answer: Not without a qualified safety assessment; this can escalate danger.

6. Is leaving always the safest immediate option?

Answer: No. The risks of leaving can differ; safety planning should be individualized.

7. What does validation mean?

Answer: Taking a disclosure seriously, avoiding blame, and respecting the survivor’s account.

8. Should confidentiality always be promised without limits?

Answer: No. Legal and safeguarding obligations may apply and should be explained.

9. Is urgent care important after neck compression even without visible marks?

Answer: Yes. Serious internal injury can occur and needs urgent medical assessment.

10. Where can people in Rwanda seek integrated GBV services?

Answer: Isange One Stop Centres; contact details should be verified with official sources.

## 14 Key terms

Term Plain-language meaning

Domestic violence Abuse occurring in a domestic or family relationship

Intimate partner violence Abuse by a current or former intimate partner

Coercive control Pattern of threats, restrictions and monitoring that reduces autonomy

Survivor-centred Support that prioritizes dignity, choices, safety and needs

Safety planning Individualized consideration of actions and resources to reduce risk

Referral Connecting someone with suitable professional support or services

## 15 Key messages to remember

- Abuse is not limited to physical injury; controlling behaviors and threats matter.
- People affected by violence deserve dignity, support, and freedom from blame.
- Leaving, disclosure, and reporting decisions may involve significant safety risks.
- The first task of a supporter is to listen, validate, consider safety, and explain options.
- Medical treatment, child protection, law enforcement, and psychosocial care have different roles.
- Rwanda has official services, but individual referral details must be checked and kept current.

## 16 References and source notes

[1] World Health Organization. Violence against women. Fact sheet, 18 June 2026. https://www.who.int/news-room/fact-sheets/detail/violence-against-women

[2] World Health Organization. Health care for women subjected to intimate partner violence or sexual violence: a clinical handbook. 2014. https://www.who.int/publications/i/item/WHO-RHR-14.26

[3] WHO, UNFPA and UNHCR. Clinical management of rape and intimate partner violence survivors: developing protocols for use in humanitarian settings. 2020. https://www.who.int/publications/i/item/9789240001411

[4] World Health Organization. RESPECT women: preventing violence against women, 2nd edition. 2025. https://www.who.int/publications/i/item/9789240117020

[5] Rwanda Ministry of Gender and Family Promotion (MIGEPROF). Directorate General of Family Promotion and Child Protection. https://www.migeprof.gov.rw/family-promotion-and-child-right-protection

[6] Rwanda Investigation Bureau (RIB). Contact Us. https://www.rib.gov.rw/contact-us

[7] UNHCR Rwanda. Gender-Based Violence: response services and Isange One Stop Centres. https://help.unhcr.org/rwanda/services/sgbv/

## 17 Editorial and professional review before publication

This document is a complete reader-facing educational draft, not an approved intervention protocol. Before public release, obtain review from a qualified GBV/safeguarding professional, a clinician for health content, and an appropriate Rwanda legal or referral specialist. Verify local service operating hours, confidentiality and reporting procedures, terminology, accessibility, and Kinyarwanda and French adaptations. Keep a record of the reviewer, approval date, changes and next review date.

Next chapter: GBV Chapter 06 - Sexual Violence, Harassment, and Exploitation. Develop it separately after Chapter 05 editorial review, with special medical, child safeguarding, and legal input.